CHRONIC HYPERTENSION — Clinical One-Pager

Sustained BP ≥130/80 (ACC/AHA) · 2017 + 2025 AHA/ACC · IM resident reference

1 · WHO TO TREAT & TARGET

Goal <130/80 for most adults (2025 AHA/ACC). Confirm dx with 2+ readings, in/out of office, proper technique. Benefit scales with baseline risk (NNT lower than statins/ASA/DM drugs).

Stage (BP)	Action
Elevated 120-129/<80	Lifestyle
Stage 1 130-139/80-89	CVD/DM/CKD or 10-yr risk **≥7.5%** (PREVENT) → drug; else lifestyle 3-6 mo then drug if still ≥130/80
Stage 2 ≥140/90	Start **2 agents** (single-pill preferred)

2025 changes: uniform <130/80 target; **PREVENT** calc replaces Pooled Cohort Eqns; stage-2 two-drug start.

2 · FOUR FIRST-LINE AGENTS

Agent	Best candidate / caution
DHP-CCB amlodipine 5-10	Most pts; good late CKD; **pedal edema; avoid HFrEF**
ACEi lisinopril 5-40	CVD, proteinuria/DM; **BMP 1-2 wk (GFR↓<30% ok), angioedema**
ARB losartan 25-100	ACEi-intolerant (cough/angioedema)
Thiazide-like chlorthalidone 12.5-25	Preferred > HCTZ; **↓Na/K, gout, GFR<30 less eff**

Never ACEi + ARB. Preferred combo: **ACEi + DHP-CCB** (ACCOMPLISH > ACEi+thiazide).

3 · RESISTANT HTN

Uncontrolled on ACEi/ARB + CCB + thiazide-like at adequate doses:

- Add **MRA = spironolactone** if eGFR ≥45 & K ok
- Then: amiloride, BB, alpha-blocker, central, **aprocitentan** (endothelin antagonist), or vasodilator
- Obesity-driven: **GLP-1 RA** for weight + BP

4 · LIFESTYLE (foundational)

Change	BP effect
Na <2 g/d	↓~5 / 2.5
High-K diet (no CKD)	↓~5 / 3
Weight loss	↓~0.5-2 / kg
Aerobic 90-150 min/wk	↓~4-6 / 3
Limit alcohol	≥2/d (F), ≥3/d (M) ↑ HTN

DASH diet overall; resistance exercise also helps.

5 · SECONDARY CAUSES

Suspect if: severe/resistant; acute or labile; age <30 / pre-puberty w/o obesity-FHx; electrolyte abnormalities.

Cause	Clue / test
Hyperaldo (most common, 20-30%)	Unexplained ↓K; **aldo:renin >20** off spironolactone
Renovascular	>50% GFR↓ post-ACEi; renal US-Doppler, CTA, MRA
OSA	Obese, snoring, +STOP-BANG/Epworth; polysomnography
Other endocrine	Cushing / pheo / thyroid / OCP; cortisol-dex suppression, TSH, metanephrines

PEARLS

- ACEi lower BP somewhat less in Black patients — still use if compelling indication (e.g. diabetic nephropathy)
- Chlorthalidone > HCTZ (potency, half-life) — more ↓K
- SGLT2i lower SBP ~5 mmHg; not FDA-approved for BP alone
- White-coat HTN = higher-risk pre-HTN state — don't treat empirically; manage other risk factors & monitor
- >20/10 above goal → expect to need 2 agents